struggles of policymakers in managing the private and public elements of health management.

There are countries that consider health a completely or largely private matter and therefore to be delivered by private-sector health providers and paid for from the incomes of individuals/families or by buying insurance. On the other hand, there are countries that consider health an entirely public concern and build a large public-sector health system to provide services to citizens. Most countries, of course, lie amid these two extremes, with a mix of public and private-sector contributions.

India has built a large public-sector healthcare system from villages to district levels—a network of health sub-centres, primary health centres (PHCs), community hospitals at selective block levels and district hospitals —funded entirely by public resources. We have also built top-class national public healthcare institutions like the All India Institute of Medical Sciences (AIIMS). Further, the country has launched a large publicly funded health insurance programme called Ayushman Bharat to take care of the medical needs of about 50 crore poor Indians.

In the private domain, India has developed a large network of hospitals and health clinics in the country. Most sophisticated medical services are available in private hospitals, in addition to top-end public institutions like AIIMS and state-level speciality hospitals.

The sector is a very large employer with doctors, paramedics and other healthcare workers.

Size of India's Health Economy

The National Accounts provide data on the output generated, inputs used and GVA by 'health and social work', which is mainly health services. In 2018–19, the value of output produced by the health and social work sector was assessed at Rs 3.16 lakh crore at constant 2011–12 prices and Rs 4.70 lakh crore at current prices. Inputs used by the sector were Rs 1.20 lakh